

DEPARTMENT OF HUMAN SERVICES

Behavioral Health

BEHAVIORAL HEALTH ADMINISTRATIVE RULES

2 CCR 502-6

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

Chapter 1: General Statutory Authority and Definitions

1.1 Statutory Authority

- A. Pursuant to Section 27-50-107, C.R.S., the State Board of Human Services, created pursuant to Section 26-1-107, C.R.S., is the type 1 board for promulgating, revising, and repealing BHA rules.
- B. Pursuant to Section 27-50-107, C.R.S., BHA may promulgate rules as necessary to implement Behavioral Health Administrative Service Organizations and care coordination services.

1.2 Definitions

- A. "Behavioral Health Administration" or "BHA" as defined in Section 27-50-101(2), C.R.S., means the Behavioral Health Administration established in Section 27-50-102, C.R.S.
- B. "Behavioral Health Administrative Services Organization" or "BHASO" means a regionally based behavioral health organization that administers and maintains adequate networks of behavioral health safety-net services and care coordination as described in Sections 27-50-301 through 304, C.R.S.
- C. "Behavioral Health Entity" has the same meaning as defined in section 27-50-101(4), C.R.S.
- D. "Commissioner" as defined in Section 27-50-101(9), C.R.S., means the commissioner of the Behavioral Health Administration appointed pursuant to Section 27-50-103, C.R.S.

Chapter 2: Behavioral Health Administrative Services Organization Requirements

2.1 Statutory Authority

- A. Pursuant to Section 27-50-107, C.R.S., BHA may promulgate rules as necessary to implement Behavioral Health Administrative Service Organizations and care coordination services.

2.2 Form and Manner to Apply to be a Behavioral Health Administrative Services Organization (BHASO)

- A. Pursuant to Section 27-50-402, C.R.S., BHA shall determine, by rule, the form and manner a qualified public or private corporation; for-profit or not-for-profit organization; or public or private agency, organization, or institution may apply to be a Behavioral Health Administrative Services Organization (BHASO).

- B. At least once every five (5) years any qualified public or private corporation; for-profit or not-for-profit organization; or public or private agency, organization, or institution may apply through a competitive bid process pursuant to the Colorado Procurement Code, Colorado Revised Statutes, Articles 101 to 112 of Title 24, to apply to be a Behavioral Health Administrative Services Organization.
- C. The commissioner shall select a BHASO based on factors of selection that include:
 - 1. The applicant's experience working with publicly funded clients, including expertise in treating priority populations determined by BHA;
 - 2. The applicant's experience working with and engaging relevant stakeholders in the service area, including behavioral health providers, state and local agencies, and the local community, including advocacy organizations and clients of behavioral health services;
 - 3. The extent to which real or perceived conflicts of interest between the applicant and behavioral health facilities or behavioral health providers are mitigated; and
 - 4. The extent to which the applicant's board complies with conflict of interest policies, including the following:
 - a. The board shall not have more than fifty percent (50%) of contracted providers as board members;
 - b. Providers who have ownership or board membership in a BHASO shall not have control or decision-making authority in the establishment of provider networks; and
 - c. An employee of a contracted provider of a BHASO shall not also be an employee of the BHASO unless the employee is the clinical officer or utilization management director of the BHASO. If the individual is also an employee of a provider that has board membership or ownership in the BHASO, the BHASO shall develop policies approved by the commissioner to mitigate any conflict of interest the employee may have.
 - 5. The extent to which the applicant's board membership reflects the diversity and interests of relevant stakeholders, including, but not limited to, representation by individuals with lived behavioral health experience and family of individuals with lived behavioral health experience.

Chapter 3: Administration and Implementation of the System of Care for Children and Youth

3.1 Statutory Authority

- A. Pursuant to section 27-64.5-102(2), C.R.S., BHA is required to promulgate rules for the administration and implementation of the system of care for children and youth.

3.2 Definitions

- A. "Child" has the same meaning as defined in section 19-1-103(21), C.R.S.

- B. “Child with Serious Emotional Disturbances (SED)” means a child or youth that has a mental, behavioral or emotional disorder, which: (1) has been diagnosed through the current diagnostic and statistical manual of mental disorders, as incorporated by reference below, the DC 0 to 5, as incorporated by reference below, or international classification of diseases, as incorporated by reference below, by a licensed mental health professional, and (2) has experienced functional impairments, which interferes with the child’s functioning in family, social relationships, school, or community, and (3) experienced the emotional disturbance over the past twelve (12) months prior to application on a continuous or intermittent basis, as determined and documented by a licensed mental health professional. SED shall not include a primary diagnosis of substance-related disorders, or primary conditions or problems classified in the DSM as “other conditions that may be a focus of clinical attention.” DSM: the diagnostic and statistical manual of mental disorders, fifth edition, text revised (DSM-5-TR) (2022) is hereby incorporated by reference. No later editions or amendments are incorporated. The DSM-5-TR is available online with an account at <https://psychiatryonline.org/doi/book/10.1176/appi.books.9781615375363> and is also available for public inspection and copying at the Colorado Department of Human Services, Behavioral Health Administration, 710 S. Ash St., C140, Denver, Colorado 80246, during regular business hours. DC 0-5: The Diagnostic Classification of Mental Health and Developmental Disorders of Infancy and Early Childhood (*DC:0-5; Zero to Three, 2016*) is hereby incorporated by reference. No later editions or amendments are incorporated. It is available for public inspection and copying at the Colorado Department of Human Services, Behavioral Health Administration, 710 S. Ash St., C140, Denver, Colorado 80246, during regular business hours. Additional resources may be found at <https://www.zerotothree.org/our-work/learn-professional-development/dc0-5-manual-and-training/> International Classification Of Diseases: The International Classification Of Diseases, 11th Revision (ICD-11) (2022) is hereby incorporated by reference. No later editions or amendments are incorporated. The ICD-11 is available online at <https://icd.who.int/en/> and is also available for public inspection and copying at the Colorado Department of Human Services, Behavioral Health Administration, 710 S. Ash St., C140, Denver, Colorado 80246, during regular business hours.
- C. “Colorado Child and Adolescent Needs and Strengths Tool (CANS)” is an evidence-based, validated, decision-support tool designed to identify the strengths and needs of children, youth, and their families and facilitate the linkage between the process and the design of individualized service plans including the application of evidence-based practices. The CANS supports level-of-care decision-making, facilitates quality improvement initiatives, and allows for the monitoring of service outcomes. Incorporated by reference CANS: the Colorado Child Adolescent Needs and Strengths (CANS) tool (2023) is hereby incorporated by reference. No later editions or amendments are incorporated. The CANS tool is available online at <https://sites.google.com/state.co.us/cans/home> and is also available for public inspection and copying at the Colorado Department of Human Services, Behavioral Health Administration, 710 S. Ash St., C140, Denver, Colorado 80246, during regular business hours.
- D. “Complex Trauma” means exposure to either multiple or prolonged traumatic events, often invasive and interpersonal in nature, or a single episode traumatic experience, that have a profound and prolonged impact on normal emotional, neurological, or behavioral development, such as witnessing the death of a caregiver; or physical, sexual, or emotional abuse or neglect resulting in the individual’s loss of a developmentally appropriate sense of a well-ordered and safe environment.
- E. “Enhanced Standardized Assessment” means an assessment process that includes a biopsychosocial assessment and the use of child and adolescent needs and strengths tool (CANS), as incorporated by reference in section 3.2(c), used to make recommendations regarding the appropriate level of care and inform treatment planning necessary for children and youth with complex and unmet behavioral health treatment needs, developed pursuant to section 27-64.5-102(1)(a), C.R.S.
- F. “Youth” has the same meaning as defined in section 19-1-103(145), C.R.S.

3.3 Eligibility and Determination of Eligibility

- A. The enhanced standardized assessment shall be administered to determine eligibility for participating in the system of care services.
1. All personnel conducting the enhanced standardized assessment shall be certified by BHA to complete the enhanced standardized assessment.
 2. The Colorado Child and Adolescent Needs and Strengths tool (CANS), as incorporated by reference in section 3.2(c), must be administered as part of the enhanced standardized assessment.
 3. All personnel conducting the enhanced standardized assessment shall maintain CANS certification.
 4. All personnel conducting the enhanced standardized assessment shall use the BHA-designated enhanced standardized assessment and CANS forms, as incorporated by reference in section 3.2(c).
- B. The populations eligible for the system of care components established by section 27-64.5-102(1), C.R.S. shall include individuals that are under twenty-one (21) years old, determined eligible through the enhanced standardized assessment process established in section 27-64.5-102(1)(a), C.R.S., and may meet at least one of the following criteria:
1. Meets the criteria to be considered a child with serious emotional disturbances (SED);
 2. Is experiencing complex trauma as determined and documented by a licensed mental health professional;
 3. Dual diagnosed with a mental health condition and a substance use disorder and/or intellectual/developmental disability;
 4. At risk of a psychiatric hospitalization due to serious emotional or behavioral health challenges;
 5. At risk of placement outside the home due to serious emotional or behavioral health challenges; and/or
 6. Is involved with one or more youth-serving systems or at risk of involvement in one or more youth-serving systems, including, but not limited to:
 - a. Crisis services
 - b. Emergency services
 - c. Juvenile justice system
 - d. County department of human or social services
 - e. Special education system

3.4 Requirements for Residential Treatment Providers

- A. Residential treatment providers participating in the system of care for children and youth must obtain cultural competency related to the provision of services by requiring applicable treatment staff to receive certification from the residential child care provider training academy established in section 26-6-923, C.R.S.

Chapter 4: Children and Youth Mental Health Treatment Act

4.1 Definitions

- A. “Ability to pay” means the amount of income and assets of the legally responsible person(s) available to pay for the individual cost of Children and Youth Mental Health Treatment Act funded services.
- B. “Care Management”, as defined in section 27-67-103(1.5), C.R.S., includes, but is not limited to, consideration of the continuity of care and array of services necessary for appropriately treating a child or youth and the decision-making authority regarding the child's or youth's placement in and discharge from behavioral health services.
- C. “Child at Risk of Out-of-Home Placement” means a child or youth who, although not otherwise categorically eligible for Medicaid, meets the following criteria:
 - 1. Has been diagnosed as a person with a mental health disorder, as defined in section 27-65-102(22), C.R.S. or experiencing negative social determinants of health as documented by a Z code in ICD-10-CM, as incorporated by reference below, and the diagnostic statistical manual of mental health disorders, as incorporated by reference in section 3.2(b); International Classification Of Diseases: The International Classification Of Diseases, 11th revision (ICD-11) (2022) is hereby incorporated by reference. No later editions or amendments are incorporated. The ICD-11 is available online at <https://icd.who.int/en/> and is also available for public inspection and copying at the Colorado Department of Human Services, Behavioral Health Administration, 710 s. Ash St., C140, Denver, Colorado 80246, during regular business hours.
 - 2. The child or youth requires a level of care that is provided in a residential child care facility pursuant to section 25.5-6-903, C.R.S., or that is provided through community-based programs, and who, without such care, is at risk of unwarranted child welfare involvement or other system involvement, as described in section 27-67-102, C.R.S., in order to receive funding for treatment.
 - 3. If the child or youth is determined to be in need of placement in a residential child care facility, they shall apply for supplemental security income, but any determination for supplemental security income must not be a criterion for a child or youth to receive services pursuant to Article 67 of Title 27;
 - 4. The child or youth is a person for whom there is no pending or current action in dependency or neglect pursuant to Article 3 of Title 19; and
 - 5. The child or youth is younger than eighteen (18) years of age, but they may continue to remain eligible for services until their twenty-first (21st) birthday.
- D. “Categorically Medicaid Eligible” means a person has met the criteria for categorically needy as defined in section 25.5-4-103(4), C.R.S.

- E. "Cost of Care" includes residential and community-based services not covered by private insurance, the family, or Medicaid.
- F. "Community-Based Services" means any intervention that is designed to be an alternative to residential or hospital level of care in which the child or youth resides within a non-institutional setting and includes, but is not limited to, therapeutic foster care, intensive in-home treatment, intensive case management, and day treatment.
- G. "County Department" as defined in section 27-67-103(5), C.R.S. means the county department of human or social services.
- H. "Dependent" means a person who relies on the responsible person(s) for financial support.
- I. "Face-to-Face Enhanced Standardized Assessment" means a formal and continuous process of collecting and evaluating information about an individual for service planning, treatment, referral, and funding eligibility as outlined in 2 CCR 502-1 § 8.4, and takes place at a minimum upon a request from the responsible person(s) for funded services through the Children And Youth Mental Health Treatment Act, section 27-67-101, C.R.S. through section 27-67-109, C.R.S. Information collected through an enhanced standardized assessment establishes justification for services and funding and is sent to the responsible person(s) pursuant to section 27-67-104(1)(a), C.R.S. The child or youth must be either physically in the same room as the licensed mental health professional or interviewed remotely using video technology during the face-to-face enhanced standardized assessment. If an in-person interview is clinically indicated or requested by the responsible person(s), the face-to-face enhanced standardized assessment must occur in-person. If the governor or local government declares an emergency or disaster, telephone may be used. Telephone shall only be used as necessary because of circumstances related to the disaster or emergency.
- J. "Family Advocate" has the same meaning as provided in section 26-5-117(6)(a), C.R.S.
- K. "Family Systems Navigator" has the same meaning as provided in section 26-5-117(6)(b), C.R.S.
- L. "First-Level Appeal" as defined in section 27-67-103(8), C.R.S., means the initial process a Medicaid member is required to enact to contest a benefit, service, or eligibility decision made by Medicaid or a Medicaid managed care entity.
- M. "Funded Provider" means any entity contracted with the mental health agency to provide community-based services or residential treatment through the Children and Youth Mental Health Treatment Act.
- N. "Licensed Mental Health Professional" means a psychologist licensed pursuant to section 12-245-304, C.R.S., a psychologist candidate pursuant to section 12-245-304, C.R.S. that is supervised by a licensed psychologist, a psychiatrist licensed pursuant to section 12-240-107, C.R.S., a clinical social worker licensed pursuant to section 12-245-404, C.R.S., a marriage and family therapist licensed pursuant to section 12-245-504, C.R.S., a licensed marriage and family therapist candidate pursuant to section 12-245-504, C.R.S. that is supervised by a licensed marriage and family therapist, a professional counselor licensed pursuant to section 12-245-604, C.R.S., a licensed professional counselor candidate pursuant to section 12-245-604, C.R.S. that is supervised by a licensed professional counselor, or a social worker licensed by pursuant to section 12-245-404, C.R.S., that is supervised by a licensed clinical social worker.
- O. "Mental Health Agency" as defined in section 27-67-103(10), C.R.S., means a behavioral health services contractor through the Behavioral Health Administration serving children and youth statewide or in a particular geographic area, and with the ability to meet all expectations of Article 67 of Title 27, C.R.S.

- P. "Plan Of Care" is a BHA developed document that at a minimum includes the anticipated frequency and costs of services to be provided for the duration of the plan of care. The plan of care is also a schedule of the fees to be paid by the responsible person(s) including, but not limited to, the estimated amount of supplemental security income payable to the residential facility if awarded to the child at risk of out-of-home placement or another provider, and sliding scale fees payable to the contractor, if applicable.
- Q. "Professional Person" as defined at section 27-67-103(11), C.R.S. means a person licensed to practice medicine in this state, a psychologist certified to practice in this state, or a person licensed and in good standing to practice medicine in another state or a psychologist certified to practice and in good standing in another state who is providing medical or clinical services at a treatment facility in this state that is operated by the armed forces of the United States, the United States Public Health Service, or the United States Department Of Veterans Affairs.
- R. "Residential Treatment" means services provided by a qualified residential treatment program, as defined in section 26-5.4-102(2), C.R.S., or psychiatric residential treatment facility, as defined in section 25.5-4-103(19.5), C.R.S.
- S. "Responsible Persons" means parent(s) or legal guardian(s) of a child or youth.
- T. "Routine Situation" means a condition where the child or youth is unlikely to become a danger to themselves or others.
- U. "State Department" means the State Department of Human Services.
- V. "Urgent Situation" means a situation where it is reasonable to believe a child or youth is likely to become a danger to themselves or others.

4.2 Application for Funding from the Children and Youth Mental Health Treatment Act

- A. A responsible person(s) may apply to a mental health agency on behalf of a child or youth for mental health treatment services for the child or youth pursuant to section 27-67-104, C.R.S., if the responsible person(s) believes the child or youth is at risk of out-of-home placement.
- B. The responsible person's request for services, appeal, and objective third party review under section 27-67-104, C.R.S. may be done with the assistance of a family advocate, family system navigator, nonprofit advocacy organization, or county department.
 - 1. The mental health agency shall provide the contact information for the organization contracted with BHA to provide these services, free of charge, to the responsible person(s) before the enhanced standardized assessment occurs.
 - 2. BHA is not obligated to pay for any services provided by entities with which they do not contract.
- C. The mental health agency is responsible for evaluating the child or youth and clinically assessing the child's or youth's need for mental health services and, when warranted, to provide treatment services as necessary and in the best interests of the child or youth and the child's or youth's family.
- D. When evaluating the child or youth to determine eligibility, the mental health agency shall conduct a face-to-face enhanced standardized assessment pursuant to section 27-67-104(1)(a), C.R.S. determination of level of care need and eligibility need will be completed jointly by the mental health agency and BHA.

- E. When evaluating a child or youth for eligibility, the mental health agency shall evaluate all areas outlined in 2 CCR 502-1 § 8.4.
- F. The mental health agency is responsible for the provision of the treatment services and care management, including any community-based care, residential treatment, or any other post-residential follow-up services that may be appropriate for the child's or family's needs.
- G. A face-to-face enhanced standardized assessment and eligibility determination shall be completed within the following time periods after a request for funding has been made by a responsible person(s).
 - 1. In an urgent situation, a face-to-face enhanced standardized assessment shall be completed by the mental health agency within three (3) working days of the initial assessment request by the responsible person(s). The mental health agency shall continue to provide care management while funded services are identified and provided. If a responsible person(s) requests that the enhanced standardized assessment be completed within three (3) working days, the enhanced standardized assessment must be completed within three (3) working days.
 - 2. In routine situations, a face-to-face enhanced standardized assessment shall be completed within ten (10) working days of a responsible person's request for services through the Children and Youth Mental Health Treatment Act.
 - 3. If the mental health agency requires additional time to make a decision following an enhanced standardized assessment and the responsible person(s) agrees, then the mental health agency may take up to an additional five (5) working days. The initial extension may be extended for an additional five (5) working days at the request of the responsible person(s), but the request should take up to but no more than twenty (20) working days from the initial request. If the responsible person(s) does not agree, the notification timelines referenced above remain in effect.
- H. The mental health agency's decision shall be communicated orally and in writing to the responsible person(s) within the time allowed for the completion of the enhanced standardized assessment. Oral notice shall be made in person or using video technology with the responsible person(s) when possible.
- I. A mental health agency's written decision shall contain the following:
 - 1. Notice of the applicable criteria for mental health treatment;
 - 2. The factual basis for the decision; and
 - 3. The appeals procedures outlined in 2 CCR 502-6 § 4.4(c).

4.3 Process of Determining Ability to Pay and Adjusted Charge for Treatment Services

- A. The mental health agency shall determine the cost of care for children and youth that receive funding through the Children and Youth Mental Health Treatment Act. All insurance and other eligible benefits shall be applied first to the cost of care. A responsible person(s) who fails to utilize available insurance and other benefits for payment will nevertheless be considered as having insurance and other benefits available for payment.

- B. Every month, the mental health agency shall determine the responsible person(s) fee by calculating seven percent (7%) of the total cost of all Children and Youth Mental Health Treatment Act funded services for the responsible person(s), excluding the costs of the enhanced standardized assessment and all care management.
- C. If the responsible person(s) is unable to pay the seven percent (7%), the mental health agency shall consider the responsible person(s) "ability to pay" as defined in section 4.1, total number of dependents, the mental health needs of those dependents, all current outstanding medical liabilities, expected length of services, and the education costs for the dependents in order to adjust the responsible person(s) fee. The mental health agency shall receive approval or denial from BHA for all fee adjustments.
- D. Responsible person(s) fee shall be no less than fifty dollars (\$50) per month.
- E. Every month, the mental health agency shall collect fees directly from the responsible person(s) or monitor that the third-party provider has collected the responsible person(s) fee.
- F. The funded provider may reserve the right to take any necessary action regarding delinquent payments by the responsible person(s).
- G. The responsible person(s) shall sign a financial agreement indicating an understanding of their financial responsibilities as described, above, to be eligible for funding through the Children and Youth Mental Health Treatment Act.
- H. Within ten (10) working days after the child's admission to residential treatment, the responsible person(s) shall apply for supplemental security income (SSI) on behalf of a child approved for funding under the Children and Youth Mental Health Treatment Act.
- I. If awarded supplemental security income, the responsible person(s) shall disclose the award amount to the mental health agency.
- J. If awarded supplemental security income, it is the responsibility of the responsible person(s) to notify the social security administration immediately upon the child or youth's discharge from residential services.
- K. If awarded supplemental security income, and awarded Medicaid, Medicaid will be used to fund treatment costs while in residential treatment. The parental responsible person(s) fee, supplemental security income, all other funding sources, and the Child and Youth Mental Health Treatment Act will fund room and board.
- L. If denied supplemental security income, the Children and Youth Mental Health Treatment Act will fund room and board and behavioral health treatment services that would otherwise have been funded by supplemental security income and Medicaid.

4.4 Appeal of the Reduction, Termination, or Denial of Mental Health Services Funded by the Children and Youth Mental Health Treatment Act

- A. Except as provided below, the mental health agency shall follow the notification process as defined in section 2 CCR 502-6 § 4.2.H-I.
- B. Oral and written notification shall be given for ongoing service determinations that reduce, increase, or terminate funded services.

- C. A responsible person(s) may request an appeal of a decrease, increase, or denial of Children and Youth Mental Health Treatment Act fund services or a recommendation that a child or youth is discharged from funded services, and the following shall apply:
1. If the responsible person(s) notifies the mental health agency of a desire to appeal a decision before termination of services, BHA and the mental health agency shall continue to fund services until the appeal process below has been exhausted;
 2. The responsible person(s) shall notify the mental health agency orally or in writing within fifteen (15) working days of a written decision of a desire to appeal a decision;
 3. The mental health agency shall have two (2) working days within which to complete an internal appeal review process and communicate a decision to the responsible person(s) orally and in writing; and
 4. The mental health agency's written decision notice of action shall contain the information required in section 21.200.43, 4.2 (e), along with the process for clinical review in section 4.4, C-E below.
- D. If the mental health agency requires more than two (2) working days to complete the internal review, and the responsible person(s) is in agreement, then the mental health agency may take up to but no more than five (5) working days to complete the review.
- E. Within five working days after the mental health agency's final denial or recommendation for discharge, a parent or guardian may request an objective third party at BHA who is a professional person to review the action of the mental health agency. Such a request may be oral or in writing, but if completed orally it must be confirmed in writing and shall be made to the commissioner of BHA or BHA's ombudsman.
- F. Unless waived by the responsible person(s), said clinical review shall include:
1. A review of the mental health agency's denial of services;
 2. An evaluation of the child, done either physically in the same room as the professional person or interviewed remotely using video technology. The evaluation shall be done in person if clinically indicated or requested by the responsible person(s); and
 3. A review of the evidence provided by the responsible person(s). The responsible person(s) shall be advised of the name and credentials of the reviewing professional, as well as any mental health agency affiliations of the reviewing professional. The responsible person(s) shall have an opportunity to request an alternate reviewing professional at BHA at that time, so long as any delay caused by the request is waived by the responsible person(s).

- G. Within three (3) working days of the receipt of the request for clinical review, a decision shall be communicated orally and in writing by the professional person to the responsible person(s), BHA and the mental health agency. The written decision shall include the relevant criteria and factual basis. If the clinical review finds residential or community-based services to be necessary and that Children and Youth Mental Health Treatment Act funding is necessary, the mental health agency shall provide services to the child within twenty-four (24) hours of the said decision. If mental health treatment providers are not available within twenty-four (24) hours and community-based services are recommended, the state level review must recommend appropriate alternatives, up to and including emergency hospitalization, if appropriate. If residential treatment is not available within twenty-four (24) hours and placement in residential treatment is recommended, the state level review must recommend appropriate alternatives, including emergency hospitalization, if appropriate, if the child is in need of immediate placement out of the home.
- H. The decision from the objective third party, at BHA who is a professional person, shall constitute a final agency action for funding through the Children and Youth Mental Health Treatment Act.

4.5 Third Party Review Process for a Medicaid Child or Youth

- A. The responsible person(s) of a Medicaid child or youth who is at risk of out-of-home placement may request, within five (5) days after all first-level Medicaid appeals processes are exhausted, an objective third party at BHA who is a professional person to review the service request made to Medicaid. The review must be conducted by a professional person as outlined in section 21.200.45 4.4 within three (3) working days of the date of request. This review does not obligate funding of services.

4.6 Dispute Resolution Process Between County Departments and Mental Health Agencies

- A. If a dispute exists between a mental health agency and a county department regarding whether mental health services should be funded under the Children and Youth Mental Health Treatment Act or by the county department, one or both may request BHA to convene a review panel consisting of family advocates, a representative from the State Department's Division Of Child Welfare, a representative from BHA, an independent mental health agency if available, a professional person, and an independent county department to provide dispute resolution. BHA shall obtain documentation from independent agencies and individuals that no conflict of interest exists pertaining to the specific child/youth being reviewed.
- B. The request to invoke the dispute resolution process shall be in writing and submitted within five (5) calendar days of either the mental health agency or county department recognizing a dispute exists.
- C. The written request for dispute resolution shall include at a minimum, the following information:
1. The county department and mental health agency involved in the dispute, including a contact person at each;
 2. The child's or youth's name and age;
 3. The responsible person(s) address, phone number, and e-mail address;
 4. Pertinent information regarding the child/youth including, but not limited to, medical or mental health status/assessment;
 5. The reason for the dispute, any efforts to resolve the matter locally, and any pertinent information regarding the child/youth;

6. Information about the child's/youth's mental health status pertaining to the dispute; and
 7. The responsible person(s) perspective on the matter, if known.
- D. BHA shall provide written notice to both the mental health agency and a county department that BHA will convene a review panel to resolve the dispute.
 - E. Each side will have an opportunity to present its position to the review panel. Interested parties will be allowed to present written or oral testimony at the discretion of the review panel.
 - F. The review panel shall have five (5) working days to complete the dispute process and issue its determination in writing to the disputing county department and mental health agency and the responsible person(s). The review panel's decision shall constitute a final decision which binds the agency determined responsible for the provision of necessary services.
 - G. If the panel deems that neither the mental health agency nor the county department is responsible for the provision of funding for the treatment of the child, then the panel shall provide a rationale for their determination. The panel shall offer recommendations for other funding sources and treatment modalities.

4.7 Responsibilities of Mental Health Agencies

In addition to the requirements and/or responsibilities established through applicable law, the mental health agency shall provide Children and Youth Mental Health Treatment Act funded services to children and youth who are eligible as outlined in Chapter 4 and the Children and Youth Mental Health Treatment Act.

- A. Children and Youth Mental Health Treatment Act services include, but are not limited to:
 1. Clinical behavioral health assessments completed by a licensed mental health professional;
 2. Community-based services;
 3. Care management services;
 4. Coordination of residential treatment services; and
 5. Non-residential mental health transition services for children and youth.
- B. The mental health agency shall provide Children and Youth Mental Health Treatment Act eligibility, service, and financial information to BHA in an agreed upon format.
- C. The mental health agency shall report information to the BHA as required and outlined in section 27-67-105, C.R.S.
- D. The mental health agency shall determine the fee for the responsible person(s) and submit the financial agreement to BHA for state approval.
- E. The mental health agency shall submit all eligibility assessments to BHA before funding approval or denial.

- F. If a child has been determined eligible under the Children and Youth Mental Health Treatment Act, the mental health agency shall submit a plan of care for approval to BHA before providing services. If necessary services are not immediately available, the mental health agency shall submit an alternative plan of care and provide interim services as appropriate.
- G. The mental health agency shall maintain a comprehensive clinical record for each child receiving services through Children and Youth Mental Health Treatment Act funding consistent with 2 CCR 502-1 §§ 4.3-4.4. Such records shall be made available for BHA review. The plan of care in the clinical record shall reflect any services provided directly by the center, including any care coordination, as defined in 2 CCR 502-1 § 1.2, services provided, and relevant documentation submitted by a third-party provider. The goal of those care management services may be, at least in part, to oversee the delivery of services by third party providers to assure that adequate progress is achieved and may reference the state plan of care and the provider's clinical service plan.

Editor's Notes

History

New rule eff. 05/01/2023.

Entire rule eff. 08/01/2025.